

Insomnia in Adults Aged 55 and Over (Circadin)

Patient Group Direction, version 003, issued 9 September 2026. Circadin 2mg prolonged-release melatonin tablets. Short-term treatment of primary insomnia, 55 years and over.

What changed, and why the service is smaller

- **THE UNLICENSED MELATONIN ARM IS GONE.** Version 001 supplied "melatonin 1mg, 3mg, 5mg tablets (unlicensed preparations)". The Human Medicines Regulations 2012 do not permit an unlicensed medicine to be supplied under a PGD at all, so that arm never had a lawful basis. It could not be corrected, only removed.
- **THE AGE FLOOR IS NOW 55, WHICH IS THE LICENCE.** It is not: Circadin is licensed from 55. Every patient aged 18 to 54 supplied under version 001 was supplied off-label without being told, because the document asserted a licence it does not have.
- **Off-label supply under a PGD is permitted where it is clearly stated and justified.** That is not what version 001 did, and rather than paper over it with a consent form, this version is written entirely within the licence. Patients under 55 are referred.
- **MAXIMUM 13 WEEKS.** That is the licensed duration and it is now the hard cap.
- **PRIMARY INSOMNIA ONLY.** Insomnia with an identifiable cause is not treated here, because melatonin will mask it. See the exclusions.

What primary insomnia means, and why it matters here

Primary insomnia is poor sleep that is not explained by another condition, another medicine, or a substance. Circadin is licensed for primary insomnia characterised by poor quality of sleep, and for nothing else.

- **Insomnia is very often secondary: to depression or anxiety, to pain, to obstructive sleep apnoea, to restless legs, to nocturia in prostatic disease, to alcohol, to caffeine, to shift work, or to a medicine the patient is already taking.**
- **Supplying melatonin to those patients does not treat anything. It delays the diagnosis, and in obstructive sleep apnoea and in depression that delay carries real risk.**
- **Take a proper history before supplying. If a cause is apparent, refer instead. That referral is worth more to the patient than the tablets.**

Circadin 2mg prolonged-release tablets

Patient Group Direction for the supply of Circadin 2mg prolonged-release melatonin tablets as monotherapy for the short-term treatment of primary insomnia characterised by poor quality of sleep, in patients aged 55 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for Circadin, the BNF and NICE CKS on insomnia.
- ALL USERS MUST BE ABLE TO DISTINGUISH PRIMARY FROM SECONDARY INSOMNIA and must take a history sufficient to do so, covering mood, pain, snoring and daytime sleepiness, alcohol, caffeine, shift work and the patient's full medicine list.
- All users must be able to screen for obstructive sleep apnoea and know that daytime sleepiness with snoring is a referral, not a supply.
- All users must be competent to deliver sleep hygiene advice as the first-line intervention, and to explain that it works better than the tablets over time.
- All users must know that Circadin is licensed from 55 years, and that this PGD does not authorise supply below that age on any basis.
- All users must previously have supplied medicines under a PGD.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to this medicine.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Monotherapy for the short-term treatment of primary insomnia characterised by poor quality of sleep, in patients aged 55 years or over, in accordance with the current SPC for Circadin. Supply is within the marketing authorisation in every respect; this PGD does not authorise any off-label use.
Inclusion criteria	• Aged 55 years or over. There is no upper age limit. • Poor quality of sleep of at least 4 weeks' duration, affecting daytime functioning. • A history has been taken and no secondary cause is apparent. See the exclusions. • Sleep hygiene advice given and, where the patient has not already tried it, a period of trying it agreed before or alongside supply. • Not currently taking any hypnotic, sedative or other treatment for insomnia. • Valid informed consent obtained. • No exclusion criterion present.
Exclusion criteria: age and licence	EXCLUDE anyone under 55 years. Refer to the GP. • Circadin is licensed only from 55 years. Safety and efficacy in those aged 0 to 18 have not been established, and the licence does not extend to 18 to 54 either. • Do not supply under 55 on an off-label basis under this PGD. If a patient aged 18 to 54 needs melatonin, that is a prescriber decision. • Tell the patient plainly why: the licence is based on the age-related fall in the body's own melatonin, which is why the evidence sits in the older group.
Exclusion criteria: secondary insomnia. Refer, do not supply	Where any of these is present the insomnia is likely secondary. Treating it here masks the cause. • Low mood, loss of interest, anxiety, or any current or suspected mental health condition. Early morning waking with low mood is depression until proved otherwise.

	● Snoring with daytime sleepiness, witnessed apnoeas, or morning headache. Suspect obstructive sleep apnoea and refer. This is the exclusion most often missed and it carries cardiovascular and road-traffic risk. ● Pain of any cause that wakes the patient or prevents sleep. ● An urge to move the legs at night, or leg discomfort relieved by movement. Suspect restless legs syndrome. ● Waking repeatedly to pass urine. Consider prostatic disease, diabetes or heart failure and refer. ● Shift work, or a sleep pattern driven by work or travel rather than by an inability to sleep. ● Alcohol used to get to sleep, or any pattern of harmful drinking. Alcohol also reduces the effectiveness of Circadin. ● Caffeine late in the day, or a high total daily intake, not yet addressed. ● A medicine that could be causing the insomnia, for example a corticosteroid, a beta-agonist, an SSRI or SNRI, a stimulant, or a diuretic taken in the evening.
Exclusion criteria: all other	Refer, do not supply, where any of the following applies. ● Known hypersensitivity to melatonin or to any excipient of Circadin. ● Hepatic impairment of any degree. Circadin is not recommended by the SPC: the liver is the primary site of melatonin metabolism and endogenous levels are already markedly raised in hepatic impairment. ● Autoimmune disease. The SPC does not recommend Circadin, as no clinical data exist in this group. ● Pregnancy, planning pregnancy, or breastfeeding. Not recommended in any of these by the SPC. ● Taking FLUVOXAMINE. The combination is to be avoided: fluvoxamine raises melatonin exposure roughly seventeen-fold. ● Taking a benzodiazepine, a Z-drug (zopiclone, zolpidem, zaleplon), or any other hypnotic or sedative. Circadin enhances their effect, and co-dosing with zolpidem measurably worsened attention, memory and co-ordination. ● Taking 5-methoxypsoralen or 8-methoxypsoralen. ● Rare hereditary galactose intolerance, total lactase deficiency or glucose-galactose malabsorption. Circadin contains 80mg lactose per tablet. ● Renal impairment where the pharmacist is not satisfied it is mild and stable. The effect of renal impairment on melatonin pharmacokinetics has not been studied. ● A previous course of Circadin supplied under this PGD in the last 6 months, or 13 weeks of treatment already completed. Refer for review rather than continuing. ● Insomnia present for less than 4 weeks. Give sleep hygiene advice and review; do not medicate a short-lived disturbance. ● Valid informed consent not obtained.
Cautions	DROWSINESS AND DRIVING. Circadin has a moderate influence on the ability to drive and use machines. Advise the patient not to drive or operate machinery if affected, and to be particularly careful the morning after the first few doses. Cimetidine, oestrogens including combined contraceptives and HRT, and quinolone antibiotics all raise melatonin levels by inhibiting its metabolism. Where the patient takes one of these, counsel on increased drowsiness and consider referral instead. Carbamazepine and rifampicin lower melatonin levels and may make the treatment ineffective. Smoking has the same effect. ALCOHOL SHOULD NOT BE TAKEN WITH CIRCADIN. It reduces its effectiveness on sleep, quite apart from its own effect on sleep architecture.

	Take after food. Food delays absorption and lowers the peak level, which is what the licensed instruction is designed around; taking it on an empty stomach changes the profile. Swallow whole. Crushing or chewing destroys the prolonged-release property and turns a night-long release into a single early peak.
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Where the exclusion is a possible secondary cause, name it and make the referral concrete: a patient who snores and is sleepy in the day needs a sleep apnoea assessment, and a patient waking early with low mood needs a mood assessment. Give the sleep hygiene advice in Appendix 1 regardless of whether anything is supplied, because it is the intervention with the better long-term evidence. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Circadin 2mg prolonged-release tablets (melatonin). PLGB 52348/0002.
Legal category	POM.
Dose and frequency	2mg once daily, taken 1 to 2 hours before bedtime and after food. One tablet per day; the dose is not titrated under this PGD.
Quantity to be supplied	Up to 21 tablets per supply, being three weeks. A maximum of 13 weeks of treatment in total, and no further supply under this PGD within 6 months of completing a course.
Maximum treatment period	13 WEEKS. This is the licensed maximum duration and there is no extension under this PGD. A patient still not sleeping at 13 weeks needs review, not a repeat.
Route and method	Oral. Swallow whole with water. Do not crush, chew or halve.
Storage	Do not store above 25 degrees Celsius. Store in the original package to protect from light.
Drug interactions	Avoid: fluvoxamine, and any benzodiazepine or Z-drug. Caution and consider referral: cimetidine, oestrogens including combined contraceptives and HRT, quinolones, 5- and 8-methoxypsoralen. Reduced effect: carbamazepine, rifampicin, smoking. Alcohol should not be taken with Circadin. Melatonin induces CYP3A in vitro at supratherapeutic concentrations; clinical relevance unknown. Check the current SPC and BNF against the patient's full medicine list.
Adverse effects	Uncommon: headache, migraine, dizziness, somnolence, lethargy, psychomotor hyperactivity, irritability, nervousness, restlessness, insomnia, abnormal dreams, nightmares, anxiety, abdominal pain, dyspepsia, dry mouth, nausea, rash, pruritus, night sweats, hypertension, chest pain, asthenia, abnormal liver function tests, weight increase. Rare: altered mood, aggression, agitation, depressed mood, depression, syncope, memory impairment, visual disturbance, palpitations, angina, and haematological effects including leukopenia and thrombocytopenia. Not known: hypersensitivity reactions, angioedema. Refer to the current SPC.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • AGE, confirmed as 55 or over. • How long the insomnia has been present, and how it affects daytime functioning.

	● THE SECONDARY-CAUSE HISTORY: mood, pain, snoring and daytime sleepiness, restless legs, nocturia, shift work, alcohol, caffeine, and the full medicine list, each asked and recorded. ● That sleep hygiene advice was given, and what the patient had already tried. ● Any previous Circadin supply, and the total weeks of treatment to date. ● Name, form, strength, dose, quantity and date of supply. ● That the patient was advised about drowsiness and driving, and about alcohol. ● Name and registration number of the healthcare professional supplying. ● Advice given, including advice given if the patient is excluded or declines. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Keep for 8 years.
Disposal	Advise the patient to return unused tablets to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product, together with the written sleep hygiene advice in Appendix 1.
Counselling	● One tablet a day, 1 to 2 hours before you go to bed, after food. ● Swallow it whole. Do not crush, chew or break it, or it will all be released at once instead of through the night. ● It may make you drowsy. Do not drive or use machinery if you are affected, and take particular care the morning after the first few nights. ● Do not drink alcohol with it. It makes the medicine work less well. ● It is not a sleeping tablet in the way you may be expecting. It works with your body clock and it works gradually. Expect an improvement in the quality of your sleep rather than being knocked out. ● The sleep hygiene measures matter more than the tablet over time. Keep doing them. ● This is a short course, up to 13 weeks. It is not a long-term treatment.
Follow-up and when to seek advice	See your GP rather than continuing if: ● Your sleep has not improved after 3 weeks. ● You feel low in mood, or you are waking very early and cannot get back to sleep. ● You are told you snore heavily or stop breathing in your sleep, or you are sleepy during the day. ● You develop a rash, swelling of the face, lips or tongue, or any reaction. Stop the tablets. For a routine query about the medicine, contact the pharmacy.

Appendix 1: Sleep hygiene advice, to be given to every patient

Give this whether or not you supply. It has the better long-term evidence, and a patient who gets only a tablet has been short-changed.

Timing	• Get up at the same time every day, including at weekends. Fixing the wake time matters more than fixing the bedtime. • Only go to bed when sleepy. • If you are awake for more than about 20 minutes, get up, go to another room, do something quiet and dull, and go back when sleepy.
Daytime	• Get daylight in the morning, ideally outdoors. • Take regular exercise, but not in the 3 hours before bed. • Avoid daytime naps, or keep them under 20 minutes and before mid-afternoon.
Substances	• No caffeine after midday. That includes tea, cola and some painkillers, not just coffee. • Alcohol gets you to sleep and then wakes you at 3am. It is not a sleep aid. • Nicotine is a stimulant and also lowers melatonin levels.
The bedroom	• Cool, dark and quiet. • Bed is for sleep. No screens, no work, no television. • Turn the clock away from you. Watching the time makes it worse.
The evening	• Wind down for an hour before bed. Dim the lights. • Write tomorrow's worries down earlier in the evening, so they are not waiting for you at midnight. • Do not eat a large meal late.

Appendix 2: Key references

- Summary of Product Characteristics for Circadin 2mg prolonged-release tablets, current version. PLGB 52348/0002.
- NICE Clinical Knowledge Summary: Insomnia.
- British National Formulary.
- Human Medicines Regulations 2012, for the position on unlicensed medicines and PGDs.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>

Authorisation

Version	v003
Supersedes	Version 002, 8 September 2026
Valid from date	9 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 9 September 2026
--------	--

Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 9 September 2026
-------------------	--

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v003
Date	8 September 2026
Changes	• The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • The unlicensed melatonin arm is removed. Version 001 supplied melatonin 1mg, 3mg and 5mg tablets described in the document itself as unlicensed preparations. The Human Medicines Regulations 2012 do not permit an unlicensed medicine to be supplied under a PGD at all, so that arm had no lawful basis and could not be corrected. • The age floor is 55, which is the licensed indication. Version 001 stated Circadin is indicated in patients aged 18 and over. It is licensed from 55, so every patient aged 18 to 54 was supplied off-label without being told. This version is written wholly within the licence rather than adding an off-label consent route. • Maximum treatment period capped at 13 weeks, the licensed duration, with a maximum of 21 tablets per supply and no further course within 6 months. • Secondary insomnia made an explicit set of exclusions with a required history: mood, pain, snoring and daytime sleepiness, restless legs, nocturia, shift work, alcohol, caffeine and the full medicine list. Obstructive sleep apnoea is called out as the one most often missed. • Hepatic impairment, autoimmune disease, pregnancy, planned pregnancy and breastfeeding added as exclusions, all being groups the SPC does not recommend. • Fluvoxamine, benzodiazepines and Z-drugs made exclusions rather than cautions. Fluvoxamine raises melatonin exposure roughly seventeen-fold, and co-dosing with zolpidem measurably worsened attention, memory and co-ordination. • Lactose content flagged for hereditary galactose intolerance, total lactase deficiency and glucose-galactose malabsorption. • Drowsiness and driving, and the interaction with alcohol, added to counselling and to the required records. • Instruction to swallow whole and take after food, with the reason for each, since crushing defeats the prolonged release and food is what the licensed dosing is built around. • Sleep hygiene advice added as Appendix 1, to be given whether or not anything is supplied.

Previous versions

001	Two arms: Circadin, stated as indicated from 18 years, and unlicensed melatonin 1mg, 3mg and 5mg tablets. Withdrawn 7 September 2026 on legal grounds.
------------	--

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Premises name
 Address
 Postcode:
 ODS code, if applicable

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

[illegible]

Signed on behalf of Get Real Health

Version v003, 9 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		9 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.